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Queratose

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O que é?

A queratose pilar é quando a pele sofre alterações na forma do aparecimento de bolinhas avermelhadas ou esbranquiçadas na pele, mais especificamente nos braços, pernas e nádegas. Essas bolinhas não coçam ou doem, por causa da produção excessiva de queratina, que acaba por se acumular nos folículos pilosos. Esta condição também é chamada de ceratose folicular ou pilar.

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O que causa?

- Alergias - rinite, bronquite, asma
- Intestino - fígado
- Dermatite atópica;
- Eczema;
- Ictiose vulgar;
- Mutações genéticas, especialmente do gene da filagrina;
- Doenças hereditárias, como a síndrome de Noonan;
- Obesidade;
- Diabetes.
- Resitência insulina
- Deficiência de vit. A
- Pele seca

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Review > J Dermatolog Treat. 2022 May;33(3):1231-1242.
doi: 10.1080/09546634.2020.1818678. Epub 2020 Sep 14.

Treatment of keratosis pilaris and its variants: a systematic review

Jalal Maghfour [1], Sophia Ly [1], Wasim Haidari [1], Sarah L Taylor [1], Steven R Feldman [1 2]

Affiliations + expand

PMID: 32886029 DOI: 10.1080/09546634.2020.1818678

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Abstract

Introduction: Keratosis pilaris (KP) is a common, benign skin condition of follicular hyperkeratosis. Although KP is asymptomatic, the cosmetic appearance of KP can lead to psychosocial distress among patients. New emerging treatments are increasingly being utilized. Yet, there is little to no summative data on the treatments of KP and its subtypes.

Objective: To summarize existing literature on treatments for KP and its subtypes.

Methods: A comprehensive search was performed using Pubmed/MEDLINE, Embase and Web of Science databases. The search identified 1150 non-duplicated articles, and 47 articles were included in the review. The primary outcomes measured were KP treatment type and the degree of improvement following therapy.

Findings: Our findings demonstrate that the most supported form of treatment for KP is laser therapy, particularly the QS:Nd YAG laser. Topical treatments - including Mineral Oil-Hydrophil Petrolat, tacrolimus, azelaic acid, and salicylic acid - are also effective at least for improving the appearance of KP.

Conclusion: While the measured treatment outcomes varied among studies, laser therapy appears to be the most effective form of treatment. Use of topicals also improved KP lesions.

Keywords: Keratosis pilaris; keratosis pilaris atrophicans; laser therapy; skin of color.

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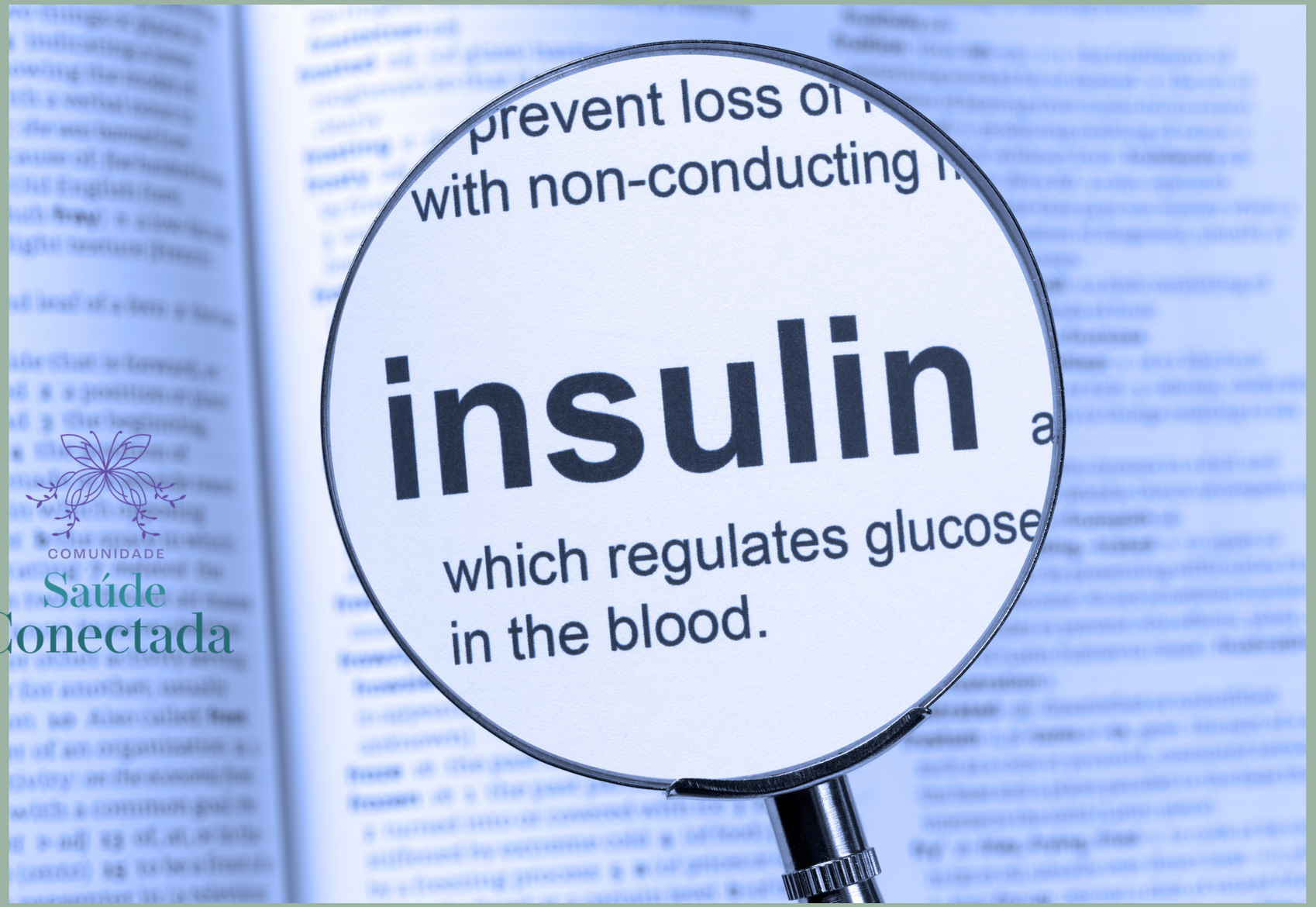

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Review > Cutis. 2021 Feb;107(2):74-78. doi: 10.12788/cutis.0173.

Cutaneous Manifestations of Nutritional Excess: Pathophysiologic Effects of Hyperglycemia and Hyperinsulinemia on the Skin

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Affiliations + expand

PMID: 33891835 DOI: 10.12788/cutis.0173

Abstract

Hyperglycemia is defined by excess blood glucose and, when persistent, may lead to prediabetic and diabetic states. Insulin is a hormone produced by the beta cells of the pancreas in response to elevated blood glucose. Dysregulated insulin secretion or clearance results in hyperinsulinemia, which also is closely associated with type 2 diabetes mellitus (T2DM) and metabolic disturbances. Hyperglycemia and hyperinsulinemia are endemic within the United States and impart considerable morbidity and mortality. Cutaneous manifestations of chronic hyperglycemia and hyperinsulinemia include acanthosis nigricans (AN), diabetic dermopathy (DD), scleredema diabeticorum (SD), ichthyosiform skin changes, acrochordons, and keratosis pilaris (KP). Necrobiosis lipoidica (NL), bullosis diabeticorum (BD), and generalized granuloma annulare (GA) are more rarely reported in association with hyperglycemia and hyperinsulinemia; however, the strength of these associations remains unclear. It is crucial for dermatologists to recognize these cutaneous manifestations, as they may be the first signs of metabolic syndrome and insulin resistance. Early identification and management of these conditions is key to improving patient health outcomes and reducing health care costs. Herein, we review the clinical presentations of these conditions and their underlying pathophysiologic mechanisms.

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PRIMEIRO PASSO

Protocolo Desinflama Conecta

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> Actas Dermosifiliogr. 2014 Mar;105(2):178-85. doi: 10.1016/j.ad.2013.09.008. Epub 2013 Nov 12.

Skin disorders in overweight and obese patients and their relationship with insulin

[Article in English, Spanish]
A Plascencia Gómez [1], M E Vega Memije [2], M Torres Tamayo [3], A A Rodríguez Carreón [2]

Affiliations + expand

PMID: 24238328 DOI: 10.1016/j.ad.2013.09.008

Free article

Abstract

Introduction: The prevalence of obesity has increased worldwide in recent years. Some authors have described skin conditions associated with obesity, but there is little evidence on the association between insulin levels and such disorders.

Objective: To describe the skin disorders present in overweight and obese patients and analyze their association with insulin levels.

Material and methods: The study included nondiabetic male and female patients over 6 years of age who were seen at our hospital between January and April 2011. All the patients were evaluated by a dermatologist, who performed a physical examination, including anthropometry, and reviewed their medical history and medication record; fasting blood glucose and insulin were also measured. The patients were grouped according to degree of overweight or obesity and the data were compared using analysis of variance or the χ(2) test depending on the type of variable. The independence of the associations was assessed using regression analysis.

Results: In total, 109 patients (95 adults and 13 children, 83.5% female) were studied. The mean (SD) age was 38 (14) years and the mean body mass index was 39.6±8 kg/m(2). The skin conditions observed were acanthosis nigricans (AN) (in 97% of patients), skin tags (77%), keratosis pilaris (42%), and plantar hyperkeratosis (38%). Statistically significant associations were found between degree of obesity and AN (P=.003), skin tags (P=.001), and plantar hyperkeratosis. Number of skin tags, AN neck severity score, and AN distribution were significantly and independently associated with insulin levels.

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Review > Cell Metab. 2012 May 2;15(5):635-45. doi: 10.1016/j.cmet.2012.04.001.

Inflammation and lipid signaling in the etiology of insulin resistance

Christopher K Glass [1], Jerrold M Olefsky

Affiliations + expand

PMID: 22560216 PMCID: PMC4156155 DOI: 10.1016/j.cmet.2012.04.001

Free PMC article

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Abstract

Inflammation and lipid signaling are intertwined modulators of homeostasis and immunity. In addition to the extensively studied eicosanoids and inositol phospholipids, emerging studies indicate that many other lipid species act to positively and negatively regulate inflammatory responses. Conversely, inflammatory signaling can significantly alter lipid metabolism in the liver, adipose tissue, skeletal muscle, and macrophage in the context of infection, diabetes, and atherosclerosis. Here, we review recent findings related to this interconnected network from the perspective of immunity and metabolic disease.

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Review > J Am Acad Dermatol. 2007 Jun;56(6):901-16; quiz 917-20.
doi: 10.1016/j.jaad.2006.12.004.

Obesity and the skin: skin physiology and skin manifestations of obesity

Gil Yosipovitch [1], Amy DeVore, Aerlyn Dawn

Affiliations + expand

PMID: 17504714 DOI: 10.1016/j.jaad.2006.12.004

Abstract

Obesity is widely recognized as an epidemic in the Western world; however, the impact of obesity on the skin has received minimal attention. The purpose of this article is to highlight the association between obesity and dermatologic conditions. We review the impact of obesity on the skin, including skin physiology, skin manifestations of obesity, and dermatologic diseases aggravated by obesity. Obesity is responsible for changes in skin barrier function, sebaceous glands and sebum production, sweat glands, lymphatics, collagen structure and function, wound healing, microcirculation and macrocirculation, and subcutaneous fat. Moreover, obesity is implicated in a wide spectrum of dermatologic diseases, including acanthosis nigricans, acrochordons, keratosis pilaris, hyperandrogenism and hirsutism, striae distensae, adiposis dolorosa, and fat redistribution, lymphedema, chronic venous insufficiency, plantar hyperkeratosis, cellulitis, skin infections, hidradenitis suppurativa, psoriasis, insulin resistance syndrome, and tophaceous gout. We review the clinical features, evidence for association with obesity, and management of these various dermatoses and highlight the profound impact of obesity in clinical dermatology.

Learning objective: After completing this learning activity, participants should be aware of obesity-associated changes in skin physiology, skin manifestations of obesity, and dermatologic diseases aggravated by obesity, and be able to formulate a pathophysiology-based treatment strategy for obesity-associated dermatoses.

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Observational Study > BMC Dermatol. 2020 Dec 9;20(1):21.

doi: 10.1186/s12895-020-00120-z.

Skin manifestations after bariatric surgery

Yada Itthipanichpong [1], Wilawan Damkerngsuntorn [1], Natsinee Tangkijngamvong [1], Suthep Udomsawaengsup [2], Patchaya Boonchayaanant [3], Chanat Kumtornrut [1], Stephen J Kerr [4], Pravit Asawanonda [1], Pawinee Rerknimitr [5]

Affiliations + expand

PMID: 33298045 PMCID: PMC7726855 DOI: 10.1186/s12895-020-00120-z

Free PMC article

Abstract

Background: Skin signs observed in morbid obesity may change as the weight reduces, especially post-bariatric surgery (BaS). Data concerning the skin findings exclusively in post-BaS patients remain limited.

Methods: Seventy post-BaS patients were examined for cutaneous abnormalities. The patients were divided into those with successful weight loss (% excessive body weight loss (EBWL) of at least 50%) and a non-successful group (%EBWL < 50%).

Results: Forty-six patients with successful weight loss demonstrated a significantly lower prevalence of acanthosis nigricans on the neck, axillae and inguinal areas, keratosis pilaris (KP) and pebble fingers. However, a higher prevalence of alopecia was observed. After adjustment with patients' factors, KP (adjusted odds ratio (aOR) = 0.21, 95%CI 0.06-0.74, p = 0.02) and pebble fingers (aOR = 0.09, 95%CI 0.01-0.89, p = 0.04) remained significantly less likely in patients with successful weight loss. Laboratory results comparing pre- and post-surgery values revealed significant decreases in fasting plasma glucose, hemoglobin A1c, and triglyceride and an increase of high-density lipoproteins in both groups. However, significant decreases of liver aminotransferases (AST and ALT) were observed only in the successful group (p = 0.04, 0.003). Nonetheless, a decrease in vitamin B12 (p = 0.01) was observed in the successful group.

Conclusion: Weight loss after BaS provided an improvement for metabolic profiles. Successful weight reduction resulted in better skin improvement. However, nutritional supplements may be necessary.

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Avaliar absorção

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Randomized Controlled Trial > J Drugs Dermatol. 2018 Mar 1;17(3):285-288.

Dermal Microflora Restoration With Ammonia-Oxidizing Bacteria Nitrosomonas Eutropha in the Treatment of Keratosis Pilaris: A Randomized Clinical Trial

Nicole Y Lee, Omer Ibrahim, Shilpi Khetarpal, Mariya Gaber, Spiros Jamas, Ioannis Gryllos, Jeffrey S Dover

PMID: 29537446

Abstract

Keratosis pilaris (KP) is a common skin finding that presents as follicular hyperkeratotic papules on the proximal extremities in patients with a propensity for atopy. Although often asymptomatic, the stippled appearance is cosmetically disturbing to patients and difficult to treat as current therapies are limited in availability and efficacy. Nitric oxide (NO) has been found to be essential in basic systemic and cutaneous physiologic function, specifically in terms of its anti-microbial and anti-inflammatory properties, which evolutionarily was maintained by ammonia-oxidizing bacteria (AOB). As modern hygiene practices have improved, there has been a gradual loss of cutaneous AOB and, therefore, the availability of an important source of human physiologic NO. We propose that restoring this dermal microflora with a purified strain of AOB, Nitrosomonas eutropha (D23), may reduce the overall cutaneous inflammatory state and, thus, be a potential therapeutic option for improving the cosmetic appearance of a skin condition such as KP which is often found in association with xerosis and atopic dermatitis. Clinical trial registry number: NCT03243617 J Drugs Dermatol. 2018;17(3):285-288. .

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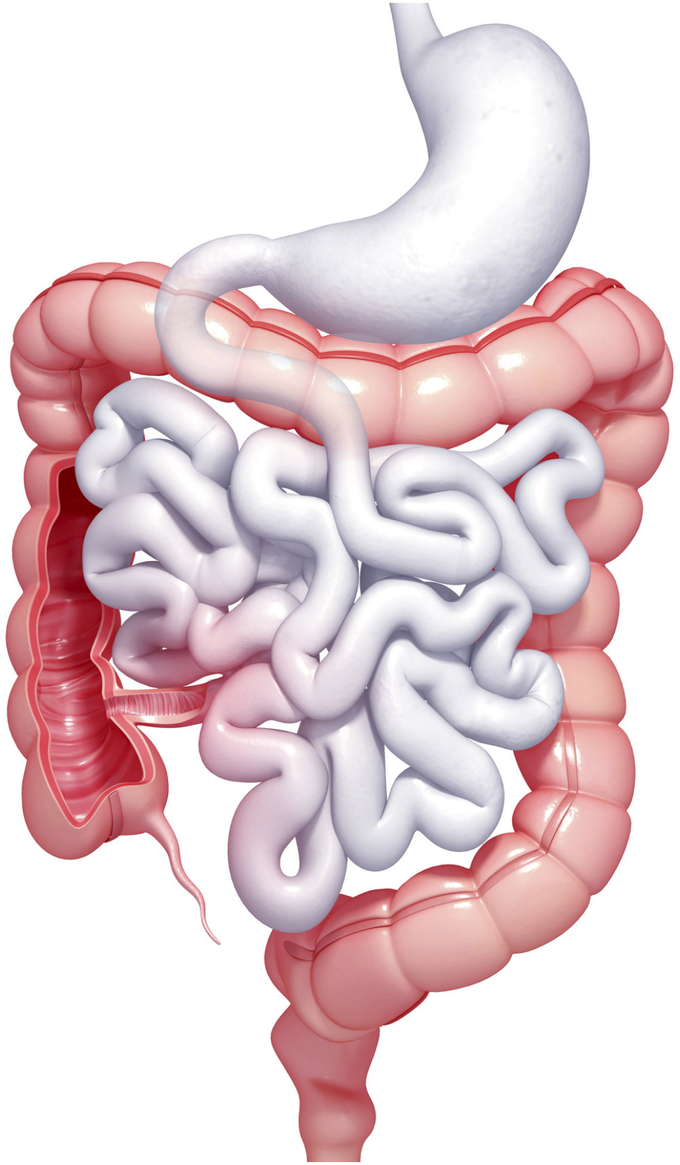

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Suplementos

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CSC

Imersão em Desinflamação

- Vit. A, C e D
- Ômega 3
- Glúten - Carboidratos
- Luteína
- OLI OLA - ext. oliveira
- Licopeno

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CSC

Mascaras naturais

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Case Reports > J Complement Integr Med. 2020 Jul 21;18(1):223-230.
doi: 10.1515/jcim-2019-0195.

Āyuṛveda management of keratosis pilaris – a case report

Rajasadanam Samsakumar Vipinsha [1], Changuli Krishna Bhat Prathibha [1], Puthanmadom Venkatraramana Sharma Anandaraman [1]

Affiliations + expand

PMID: 32692702 DOI: 10.1515/jcim-2019-0195

Abstract

Objectives: Keratosis pilaris (KP) is the condition of the skin with extensive keratin follicular plugging. It may be associated with the erythema. The upper arm extensor area, shoulders, back of neck and thighs, as well as face and the upper trunk are the areas of presentation. Available medications for KP give only symptomatic relief, while some produce serious side effects. There is no proven universal treatment for the disease that can provide complete recovery. Āyuṛveda management of KP is not yet reported.

Case presentation: A 26-year-old male patient, presented with main complaints started with papular lesions over his right shoulder, chest and upper back along and later with pustular lesions in the past 2 weeks. The condition was associated with redness, mild swelling and itching. The case was diagnosed as Keratosis pilaris based on its presentation, site, and pathogenesis. Also by analyzing the extent of vitiation of doṣas (morbidities), the Vata kapha pitta hara line of treatment was adopted, which was accomplished in two phases i. e. Śodhana Cikitsa and Śamana Cikitsa.

Conclusion: Both internal and external treatments along with diet restrictions were found effective in arresting the pathogenesis and recovery in a short period. All the symptoms associated with the condition were completely cured with no signs of re-occurrence.

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Você também pode usar óleo de coco orgânico + açafrão (2 pitadas de açafrão em 1 colher de sopa de óleo de coco)

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Mel

Com suas propriedades calmantes, o mel provou ser muito eficaz no tratamento de muitas doenças da pele. Pode ser diluído em água e aplicado na área afetada.
Isso proporciona alívio da coceira, que é muito comum nas bolhas. Isso é especialmente útil na área sob os olhos.

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Aveia

Esmagar uma xícara de aveia e adicioná-la à água do banho ajuda na limpeza da pele e remove a pele morta. Também proporciona alívio contra a irritação, acalma a pele e proporciona alívio da coceira.

Banhos de aveia coloidal são frequentemente recomendados por suas propriedades de cura da pele. A aplicação regular de pasta de aveia nas áreas afetadas ou adicioná-la ao banho pode melhorar a pele afetada pelo eczema. A aveia pode esfoliar suavemente a pele seca e remover as camadas de células mortas, ajudando no processo de renovação da pele.

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Gel de Aloe Vera

Este é um dos tratamentos naturais mais simples e eficazes para o eczema. O gel extraído diretamente da planta de aloe vera pode ser aplicado nas áreas afetadas para alívio imediato. Sua intensa capacidade hidratante hidrata a pele seca e a natureza refrescante acalma a pele afetada por inflamação, eritema e coceira.

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Aloe vera também pode ajudar a reparar a barreira da pele, enquanto suas propriedades antibacterianas ajudam a prevenir as chances de infecção.

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Calêndula e mel

Flores e folhas de calêndula são medicamentos antigos usados por suas propriedades cicatrizantes, anti-inflamatórias e antimicrobianas. Usá-lo em combinação com mel melhora ainda mais o tratamento do eczema.

Como usar calêndula e mel:

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Faça um pacote de rosto moendo 4 a 5 flores e folhas de calêndula. Adicione o mel à mistura e aplique nas áreas afetadas. Lave após 15 a 20 minutos para curar o eczema leve.

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